28 October 2026 | Evaluation plan v7 | Discussion draft — no expenditure approval
MEMORANDUM

Panda Vision: regional investment and value of perspective

Action required

Use an existing-evidence, five-configuration model to compare new investment, continuation and shared equipment. Examine who bears costs and who benefits before a local or regional investment decision. Simulated review is author self-review, not independent clinical validation. No trial, new recruitment or structured elicitation is required for this preliminary evaluation.

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Presented by	CMO / neonatal, retrieval, virtual-care and finance partners

Executive summary

1. Decisions and population. Compare telephone, mobile teleconference, Panda Vision, ceiling/wall PTZ and a complete tablet/OBSBOT kit. Consultation remains in all arms. Retain the three-site pilot and 33-site state scenario; add a 10-location CHHHS hospital/emergency network and a 14-location CHHHS + TCHHS network. These are not all community clinics or a certified deployment list. Preserve historical births and missingness; separately test reopened Cooktown/Weipa, emergency-readiness activity and Cairns as referral-only. [R7]

2. Clinical and resource model. Keep the v6 explicit immediate-transfer, delayed-rescue and final-local-care pathways, failed-video fallback and no unsupported transfer-avoidance credit in critical cases. Price hospital components once; value incremental video time separately from paid capacity. IHACPA inlier prices are prototypes, not actual coded NWAU or marginal tariffs. QEP and local QWAU adjustments remain unverified. [M6; NEP26]

3. Perspectives and decision discordance. Report health-system resources, a bounded societal extension, CHHHS resource exposure, patient/family money and time, and conditional state/Commonwealth allocation. Funding and subsidies are transfers: they must reconcile rather than create extra social savings. CHHHS receiving costs are restricted to the modelled regional catchment. Apply vop_poc_nz's fixed expected-choice directional regret with explicit ties; do not confuse it with per-draw perfect information. [VOP; M7; NHRA26]

4. Evidence and uncertainty. Audit all active and retained parameters for provenance, ranges, distribution and precision. No local statistical model is fitted to the 4–5 Atherton cases. Use shared systematic draws and separate null, sceptical, unattenuated-effect and structural scenarios. V7 completes 44 paired 20,000-draw structures plus allocation sensitivity and health-value thresholds. Keep established-only and conceptual prototype decisions distinct. [A7]

5. Health consequences. Reference QALY/DALY improvement remains unquantified. EPICure2 provides age-11 utility differences, not a causal video effect. Use separate HUI2/HUI3 bridge scenarios and calculate the health gain required to justify costs when transfers do not fall. Do not convert a disability weight to a utility by 1 minus the weight, or add QALYs and DALYs. Additional survival with morbidity is not excluded. [EPICURE2; AIHW_ABDS]

6. Decision and publication. Panda's conditional health-system cost-saving probability is 50.5% in the pilot, 46.3% in the CHHHS hospital network, 39.2% in the combined network and 65.6% in the illustrative state network. Different readiness/activity assumptions explain why scale does not guarantee value. Complete independent review, critical full-text gaps and connected release checks before submission; do not add unnecessary modelling infrastructure. [M7; V7]

Engagement and consultation

Consumers	Include family travel, time and separation. Label the societal boundary; education, wider unpaid care, lifetime disability services and environmental effects are not fully valued.
Clinicians and staff	Four simulated clinical/economic/statistical/security passes identify and adjudicate issues. They are not independent agents or professional sign-off.
External stakeholders	Ordinary records, invoices and public-source updates may replace assumptions. No vendor, network or government endorsement is implied.

Implications	
Budget	Keep economic resource costs, capacity cash schedules and funding allocation separate. The .405 Commonwealth response is an uncapped sensitivity illustration, not a marginal-payment or entitlement calculator.
FTE	No new establishment is approved. Hub time remains incremental to telephone; cash blocks depend on spare hours. Receiving-share and retrieval-payer allocations are explicit scenarios.
Risk	Clinical effect, capability, casemix, local prices and family costs remain uncertain. Two regional portfolios are readiness scenarios, not a recommended installation list.
Legal	No patient-level data added. Obtain governance, authorship and source-redistribution permission. Keep corporate templates and the ELC excerpt private.
ICT	Preserve a lean NumPy model and Polars-only DataFrame policy. Gitleaks, the full pinned stack, real lock, hosted CI and Conductor checkout remain unverified when unavailable.
Impact on First People's Health	Assess equitable access and displacement independently of cost ranking. No disability-based exclusion or claim that more survival must save money is made.

Strategic Plan alignment

Our care: appropriate neonatal support. Our people: explicit workload and usable systems. First Peoples health: equitable rural/remote access. Our sustainability: reconciled investment and perspective analysis.

Attachment/s

1. v7 audit and perspective supplement. 2. Workbook, exact tables and eight figures. 3. CHHHS slides. 4. Updated Git repository, source registry/archive, simulated reviews and actual assurance receipts. R7: roster; M7: outputs; A7: parameter audit; V7: review/assurance. Public original source files remain incompletely archived; transcriptions are not originals.